

F027



بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ

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Dr. Abdul Samad Memorial Hospital

G.DH.THINADHOO, MALDIVES. TEL:684 1977 FAX: 684 1973

Patient Record



Trauma & Emergency Medicine

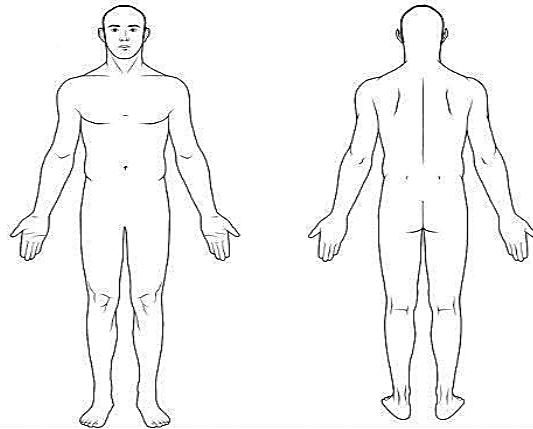
Patient Name: Zulfa Abdulla	Age: 51y	Sex: F	Arrival	Time:
Hospital No: 64997	Bed No:	NID/Passport No: A283936		Date: 13/06/26

Referred By:
Presenting Complaints: PATIENT DEVELOPED FEVER AND CHILLS AT THE START OF HD FEVER SINCE LAST NIGHT TEMP AT ARRIVAL OF HD 38C H/O WET CATHETER DRESSING H/O OOZING FROM CATHETER SITE
Past History:
Medication: TAKING TAB PARACETAMOL AT HOME
Allergy:
Last Meal:
LMP:-
Working Diagnosis: CKD - 5 ON HD (ADPKD) CATHETER RELATED INFECTION

A	Speech/ Add Sounds		
	SPO2 %	99	
	SPO2 Per Liter	RA	
B	RR /Min		
	Chest		
C	PR /Min	104	
	BP /mmHg	100/53mmhg	
	Heart Sounds	S1S2M0	
D	GRBS mg/dl		
	E		
	V		
	M		
	TOTAL		
		Right	Left
	Diameter		
	Reaction to Light		
	Corneal Reflex		
		Right	Left
	UL		
	LL		
E	Temp C	38.1C	
	Abdomen/Log roll:	Abd- full, no undue tenderness	
	DRE:		
	Bedside USG		

Physical Examination

GC-FAIR

REDDISH EXIT SITE WITH MILD DRY PUS
DISCHARGE

Initial treatment :

Inj PCM 1g IV stat

Swab C/S

Blood C/S

Blood C/P

D/W Dr Jaya (regarding removal of catheter and availability of antibiotics)

Plan: Remove perm catheter

Inj vancomycin 1.5g iv loading dose, then 1g iv post HD (modify according to culture)

Inj ceftazidime 1g iv post HD - both injections not available

Revised:

Start inj ceftriaxone 1g iv stat- developed fever and chills after inj xone

IV hydrocortisone 100mg stat

Observe in ER, temp charting,

Investigation Ordered

Blood	CBC	✓	Urine	Routine	✓
	LFT	✓		Culture	
	RFT	✓		ECG	
	Electrolytes	✓			
	Cardiac Markers		Radiology	Xray	
	CRP	✓		USG	
	Dengue	✓		Doppler	
	RBS	✓		CT	
	S.BHCG/UPT			MRI	
	Others				

Disposition

Discharged by ER:			Admitted:	
Referred:				
Discharged by referred Department				

Name of Attending Doctor: Dr Maryam for Dr Aroona

Signature: